

How should I diagnose type 1 diabetes in a child or young person?

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- **Suspect type 1 diabetes in a child or young person presenting with hyperglycaemia (random plasma glucose more than 11 mmol/L) and the characteristic features of:**
 - Polyuria.
 - Polydipsia.
 - Weight loss.
 - Excessive tiredness.
- **If type 1 diabetes is suspected**, refer the child or young person immediately (on the same day) to a multidisciplinary paediatric diabetes care team with the competencies needed to confirm the diagnosis and provide immediate care.
- **When diagnosing diabetes in a child or young person**, assume type 1 diabetes unless there are strong indications of other types of diabetes.
 - Features of type 2 diabetes include:
 - A strong family history of type 2 diabetes.
 - Obesity.
 - Black or Asian family origin.
 - No insulin requirement, or have an insulin requirement of less than 0.5 units/kg body weight/day after the partial remission phase.
 - Evidence of insulin resistance (for example acanthosis nigricans).
 - Features of other types of diabetes (such as monogenic or mitochondrial diabetes) or other insulin resistance syndromes include:
 - Diabetes in the first year of life.
 - Rarely or never develop ketone bodies in the blood (ketonaemia) during episodes of hyperglycaemia.
 - Associated features, such as optic atrophy, retinitis pigmentosa, deafness, or features of another systemic illness or syndrome.

Basis for recommendation

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